



POLICY AND PROCEDURE

Medication: Management of Controlled (Schedule 8 (S8) and Schedule 4 Restricted (S4R)) Medicines

Scope (Staff):	North Metropolitan Health Service Mental Health (NMHS MH) Staff
Scope (Area):	NMHS MH Inpatient Sites

Contents

1. Aim.....	2
2. Background.....	2
3. Risk.....	3
4. Definitions	3
5. General Information	4
5.1 Pharmacy Department Access	4
5.2 Staff Authorisations and responsibilities	4
5.3 Chain of Custody	5
5.4 Storage of Controlled Medicines.....	6
5.5 Controlled Medicine Registers/ Requisition Books/ Related Paperwork.....	6
5.6 S8 Key/ Safe Combination	6
5.7 S4R Key	6
5.8 Lost Keys.....	7
5.9 Automated Dispensing Machine/Cabinet (ADM)	7
6. Ordering, Distribution, Supply and Delivery	7
6.1 S8 Medicines.....	7
6.2 S4R Medicines	8
6.3 Telephone Orders.....	9
6.4 After-hours supply to inpatient wards	9
7. Administering	9
7.1 S8 Medicines Administration	9
7.2 S4R Medicines Administration.....	10
7.3 Dose Administration Aids (DAA).....	10
8. Dispensing and Supply	10
9. Inventory Management	10
9.1 Clinical Areas.....	10

9.2 Pharmacy Department.....	11
9.3 Controlled Liquid Medicines	11
9.3.1 Calculating a controlled liquid discrepancy	12
9.4 Emergency Drug Box/ Medicine Kit.....	12
9.5 Patient's Own Medication (POM).....	13
9.6 Discrepancies	13
10. Destruction, Discard and Disposal	14
11. Transport of S8/ S4R Medicines	15
11.1 Transport of inpatient S8/S4R Medicines – Imprest and Inpatient Supply	16
11.2 Transferring S8/S4R Medicines between Wards	16
11.3 Transport of Discharge/Leave S8/S4R Medicines	16
11.4 Transport of S8/S4R Patient's Own Medicines (POMs)	17
11.5 Transport of Emergency Medicine Kits Containing S4Rs	17
11.6 Transport of S8/S4R Outpatient Medicines	17
11.7 Transport of S8/S4R Medicines to/from other sites (Borrow/Lend).....	17
12. Day Procedure Unit (DPU).....	18
13. Exceptions to Administration/Discard in Specific Areas or Situation	18
14. Compliance and Evaluation	18
Appendix 1. Management of S8 and S4R Medicines at Ward Level.....	21
Appendix 2. NMHS Medicines Management Audit Schedule.....	22
Appendix 3: Approved Staff and Approved Transport Methods of S8/S4R Medicines.....	24
Appendix 4 Staff Authorisations for handling of S8 and S4R Medicines	25

1. Aim

To provide minimum requirements for the management of S8 and S4R medicines, ensuring patient safety and reducing the risk of diversion during the processes of ordering, supplying, distribution, administration, destruction, discarding, disposal, and discrepancy management of controlled medicines.

2. Background

The consequence of diversion, misuse and mishandling of S8 and S4R medicines, also known as controlled medicines, is high. Extra policies and procedures are required to prevent this from occurring. There are Western Australian (WA) government policies, North Metropolitan Health Service (NMHS) and NMHS MH policies and procedures to comply with.

3. Risk

Controlled (S8/S4R) medicines are more liable to be abused and dependence may lead to unauthorised use and diversion into illicit activities.

Culture	☒	Policy & Legislative Compliance	☒
Governance & Accountability	☒	Staff Safety & Wellness	☒
	☐	Quality of Patient Care	☒

4. Definitions

Administration	The preparation and giving of a medicine to a patient by any route, usually for the patient to take immediately.
Approved Staff Member	Refers to a NMHS staff member who is: • an authorised staff member, or • a staff member who is allocated the responsibility to complete a particular task related to medicines as per their job description.
Authorised Staff Member	A health practitioner who is a NMHS employee authorised by legislation to be involved in certain processes such as procurement, distribution, supply, storage, administration and/or disposal of scheduled medicines. The processes a practitioner can participate in are dependent on their individual profession and any additional training or endorsement they hold as defined in the WA Medicines and Poisons Regulations 2016.
Chain of Custody	In relation to medicines, the chronological documentation that records the transfer of a medicine from one individual/area to another. Sometimes described as the “paper trail”, even though the records may be electronically made and stored. A critical element of maintaining the chain of custody is that the records must be robust and auditable.
Controlled Substance	This term includes both Schedule 8 (S8) and (drugs of addiction; drugs of dependence) Schedule 4 Restricted (S4R) Medicines (considered to have a higher risk of diversion).
Delegation	Within the context of nursing is the action by which a RN/Midwife delegates aspects of care to another care provider who has the appropriate education, knowledge & skills to undertake the activity within their scope of practice safely, within their scope of practice.
Dispense	In relation to a medicine means supplying the medicine in accordance with a prescription duly given by a Medical Practitioner. Only a Pharmacist or Medical Practitioner may dispense medicines.
Distribution	The process of supplying medicines to an inpatient ward. An order is distributed by pharmacy staff using the iPharmacy program, electronically.
Diversion Risk	Unauthorised possession, supply, or receipt of a scheduled medicine for misuse, abuse or illicit use.
Enrolled Nurse	An Enrolled Nurse is defined as a Nurse who is registered with the Australian Health Practitioner Regulation Agency (AHPRA) as an enrolled nurse in Division 2 of the Register.
iPharmacy	Pharmacy dispensing, ordering, stocktake and distribution software program.

Medication	Defined as any substance or medication used for therapeutic or diagnostic purposes.
SHERLOC	System for Handling Evaluation Reports on Losses of Controlled medications.
S8	Medicines with a very high risk of diversion and misuse. Also referred to as 'drugs of addiction', 'controlled medicines' or 'drugs of dependence'. The list of medicines designated S8 are listed in the Standard for the Uniform Scheduling of Medicines and Poisons (SUSMP).
S4R	These are Schedule 4 (S4) medicines considered to have a higher risk of diversion and misuse. Also referred to as 'controlled medicines'. The list of medicines designated S4R are listed in the Risk Based Requirements for Medicines Handling policy
Student Nurse	This refers to a student undertaking preparation to practice as a Nurse (EN or RN). A Student nurse may administer, transport and discard S8 and S4R medicines under the direct supervision of a RN depending on their stage of education and level of competency; only the RN is authorised to sign the medication chart (Refer to NMHS Appendix 1).
Supervision	Defined as Direct Supervision – when the supervisor is present and personally observes, works with, guides, and directs the person who is being supervised.
Supply	After distribution via iPharmacy the medicine is supplied to the requesting inpatient ward. Controlled medicines from the after-hours safe/cupboard may be supplied to a requesting ward; documentation is required in the appropriate register.
Independent double-checking	Two authorised staff members separately check each component of the work process. Medication administration – 6 Rights are followed.

5. General Information

5.1 Pharmacy Department Access

The Pharmacy Department must always have the direct supervision of a pharmacist at all times; other pharmacy staff can only enter when a pharmacist is present. Only pharmacy staff have access to the pharmacy department. Access for non-pharmacy staff is only allowed once identification and reason for entry is verified.

The NMHS MH pharmacy department, based at Graylands Hospital, is not opened after-hours. Only approved pharmacists have access to the pharmacy department after-hours.

5.2 Staff Authorisations and responsibilities

Please refer to [Appendix 4](#) Authorisations of Handling of S8 and S4R Medicines for staff roles and responsibilities. The overarching person responsible is the Chief Pharmacist, who holds the permit to have authority to purchase scheduled medicines.

Administration of S8 and S4R medicines must be handled by **authorised** staff members and **approved** staff members (see Section 4: Definitions) who are suitably trained and competent in the task. Refer to [Appendix 4](#) Authorisation of Handling of S8 and S4R Medicines.

- Individual staff must also consider their personal scope of practice and professional credentialing in determining whether their handling of S8 and S4R is “the lawful practice of their profession’.
- Student nurses/midwives are permitted to administer S8 and S4R medicines if under the direct supervision of an authorised person who is a Registered Nurse/Midwife. The Registered Nurse/Midwife providing direct supervision of the student nurse/midwife is responsible for all actions of the student nurse/midwife in relation to administering S8 and S4R medicines (including access/retrieval, documentation, disposal etc.).
- Where restriction or conditions are placed on individual staff members limiting their rights to administer S8 or S4R medicines, the individual is responsible for informing their line managers of this restriction, and practise within their scope of practice.
- Enrolled Nurses work under the direct or indirect supervision of a Registered Nurse and are responsible for their actions and are accountable in the provision of delegated care (EN Standards of Practice NMBA).

5.3 Chain of Custody

Accurate and complete records of all controlled medicine transactions are to be maintained at the time of the transaction. Distribution processes are in place to ensure each staff member involved is identifiable, for each stage of the transaction.

To reduce the risk of diversion at the minimum two authorised staff must be involved in all stages of controlled medicine transactions. These include distribution, supply, the administration process, including the medication chart order review, medicine access and retrieval process, dose preparation, administration, discards, documentation, and inventory. The second authorised staff member is the independent witness for all processes involved in the transactions.

Tamper-evident bag numbers are used for all ward supplies to inpatient wards. Discharge or leave medication dispensed in the pharmacy department are sealed with a tamper-evident seal.

Ward Level

There is a requirement for two authorised staff members to be involved, one as a witness:

- from the moment of access of the medicine until the point the medicine has been administered to the patient (including discards where applicable).
- for the completion of accurate documentation of all parts of the administration process; from access to administration to disposal of medication.

Pharmacy

S4Rs: The pharmacist and pharmacy assistant (or two pharmacists) must be involved in the distribution, checking, dispensing and inventory of the medicine.

S8s: Two pharmacists must be involved in distribution, dispensing, stocktakes, destruction and receiving stock.

5.4 Storage of Controlled Medicines

Pharmacy Department (Graylands): All controlled medication, with the exception of patients own medication (POMs) are stored in the Automated Dispensing Machine/ Cabinet (ADM). See Section 5.9. S8 POMs are stored in the S8 safe, due to lack of room in the ADM. S4R POMs are stored in the locked drawers. This also includes any S4Rs dispensed for future blister-packing for patients.

Inpatient Wards: S8 safes and S4R locked cupboards store all the controlled medicines on the wards. Refer to the [NMHS MH Medication: Patient's Own Medication Policy and Procedure](#) for information on storing POMs.

5.5 Controlled Medicine Registers/ Requisition Books/ Related Paperwork

All S8 and S4R registers and requisition books are ordered from the pharmacy department are uniquely numbered and documented in a spreadsheet. Registers, requisition books and related paperwork (picking slip/ invoice) must be returned to the supplying pharmacy department.

If a register cannot be located within 24 hours of being detected as missing, a report must be made immediately to the Department of Health via email: MPRB.Compliance@health.wa.gov.au.

Records are archived and stored:

- S8 medicine records archived for 7 years if completed before 29th January 2017 or 5 years if completed after 30th January 2017.
- S4R medicine records archived for 2 years.

NMHS MH Pharmacy department will follow the pharmacy procedure - Internal and External Archiving and Disposal Schedule to record and archive controlled medicine registers and requisition books.

5.6 S8 Key/ Safe Combination

The NMHS MH Pharmacy department safe combination is only given to the Chief Pharmacist (CP), Deputy Chief Pharmacist (DCP), dispensary pharmacist and clinical pharmacists who are on the on-call duty roster. Only POMs are stored in this safe since the implementation of the Automated Dispensing Machine/ Cabinet (ADM) for controlled medicines.

The keys to access ward S8 medicines must only be held by an authorised staff member. The S8 key to the medication safe must be handed over by an authorised staff member to another authorised staff member and signed for at which point the balance of stock must be checked by the nurse receiving and the nurse relinquishing the key.

5.7 S4R Key

When not in use, the NMHS MH Pharmacy department S4R key is kept in the key safe in the DCP's office. During pharmacy opening hours, the NMHS MH Pharmacy department dispensary pharmacist is responsible for the S4R key, and it must be kept on their person. The S4R key is signed out of the S4R key register by the dispensary pharmacist at the beginning of the shift and back in at the end of the shift and before handing over to another pharmacist, when necessary.

The key to the S4R medication cupboard on the ward must be always kept on the person of a nominated authorised staff member. The key must be handed over and signed for on a shift-by-shift basis to another authorised staff member.

5.8 Lost Keys

Controlled medicine safe/cupboard/ drawer keys lost must be reported to the authorised staff. On the ward this would be the coordinator or CNS, in the pharmacy department report to the Chief or Deputy Chief Pharmacists. Refer to [NMHS MH Security and Access Control Procedure](#).

5.9 Automated Dispensing Machine/Cabinet (ADM)

The ADM replaces the physical safe and paper register for S8 and S4R transactions in Graylands Hospital Pharmacy Department. The fingerprint scanner is used to log into the device, staff must have their fingerprints registered to access the machine. All transactions require a witness, the first authorised user is the initiator, the second authorised staff member is the witness, each scan their fingerprint.

The Deputy or Chief Pharmacist receives an electronic report each month from NMHS showing all the transactions of the ADM.

If a security or data breach occurs in relation to an approved electronic register, a report must be made to the Department of Health via email: MPRB.Compliance@health.wa.gov.au within 24 hours of the security or data breach being detected.

If after-hours a controlled medicine cannot be procured from the after-hours safe/cupboard or a community pharmacy, requiring the on-call or weekend duty pharmacist to supply from Graylands Hospital pharmacy department, the pharmacist can supply the required medication without a witness. The ADM will send an automated report to the Deputy or Chief Pharmacist, that the controlled medicine has been accessed without a second witness.

6. Ordering, Distribution, Supply and Delivery

Graylands Hospital, Frankland Centre and Selby Lodge have medication supplied by the SCGH pharmacy department.

A chain of custody must be maintained for S8 and S4R medicines to reduce the risk of diversion. A minimum of 3 authorised staff members MUST be involved in the whole process.

6.1 S8 Medicines

S8 medicines use on the ward must be ordered, distributed, and supplied according to the local site Pharmacy Schedule 8 Medication Procedure. Newly charted S8 medicines must be ordered via the clinical pharmacist.

S8 medicines for use on the ward must be ordered by a nurse or a pharmacy staff member. All orders are distributed and supplied by the site's pharmacy dispensary, using the requisition book and delivery bag with a security number tag.

Graylands Hospital, Frankland Centre and Selby Lodge

S8 medicines for use on the ward can only be delivered by a pharmacy staff member.



SCGH MHU, MHOA and Osborne Lodge

S8 medicines for use on the ward can be delivered or picked up by a pharmacy staff member or two authorised nurses in accordance with local site procedures.

6.2 S4R Medicines

S4R medicines on ward imprest (except for lorazepam injection) may be ordered by nursing staff, pharmacist, or the imprest pharmacy assistant. Formulary S4R medicines supplied by Sir Charles Gairdner Hospital (SCGH) to NMHS MH Pharmacy department are delivered directly to the ward by the courier. Non-formulary S4R medicines for Graylands, Frankland and Selby sites are delivered from SCGH to NMHS MH Pharmacy.

Newly charted S4R medicines not on imprest must be ordered via the clinical pharmacist. On delivery of S4R medicines from SCGH or Osborne Park Hospital (OPH) pharmacies, **two** authorised staff members must enter the stock into the medication register and **both staff** confirm the balance of stock by signing the register.

On delivery of Restricted Schedule 4 medications two staff members (Pharmacist or Pharmacy Assistant with an authorised staff member from the ward) must enter the stock into the medication register and both staff confirm the balance of stock by signing the register. All delivery and inventory check entries must be written in **red** ink. All other entries must be made in **black** ink.

Delivery dockets and/or requisition forms are to be returned to the appropriate pharmacy department supplying the medicines, either SCGH, OPH or NMHS MH Hospital. These must include signatures and legible printed names of all staff involved in the process. Original requisition forms must be returned to the relevant pharmacy department but completed delivery dockets can be returned to the pharmacy department electronically.

Once supplied, the tamper-evident seal on stock must be checked to ensure it is intact, then the quantity received is entered into the appropriate register, balance of stock confirmed and signatures and printed names of the two staff members involved in the process entered in the register.

Refer to the [NMHS Order and Receipt of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy](#); [NMHS Supply and Distribution of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy](#)

Alterations, obliterations, or cancellations shall not be made in the S8 or S4R medication registers. Any mistake made in any entry must be corrected by means of a footnote, initialled and dated.

Lorazepam Injection

Lorazepam 4mg/1mL solution for injection has been registered in Australia; this is the product of choice for NMHS MH.

Lorazepam 4mg/1mL is subjected to S4R restrictions and must be stored in locked boxes in the pharmacy and ward refrigerators (+2-+8°C).

Osborne Lodge orders and receives lorazepam IM from the OPH Pharmacy.

SCGH Mental Health Unit (MHU) and SCGH Mental Health Observational Area (MHOA) order and receive lorazepam IM from the SCGH Pharmacy.

Propofol

Management of propofol will be in accordance with the [NMHS Management of Propofol Policy](#).

6.3 Telephone Orders

Record the order on the 'Telephone Order' section on the front of the WA Hospital Medication Chart (HMC) PMR 60A.

- Phone or verbal orders have a greater risk for error.
- S4R telephone orders, must be taken by an authorised staff member and a second authorised staff member must confirm the Medical Practitioner's (MP) order.
- Initials of the two nurses taking and confirming the order must be on the chart.
- **The MP must sign the chart within 24 hours.**
- Refer to the [NMHS MH Medication: Prescribing, Administration and Management for Inpatients Policy and Procedure](#) for information on taking telephone orders.

Telephone orders **MUST NOT** be taken for S8 medications.

6.4 After-hours supply to inpatient wards

Refer to the inpatient site/s imprest list to search for the available medicine. If not available, follow the processes for obtaining from another source as per the [NMHS MH Medication: After-Hours Supply Policy](#).

Prescriptions written for S8 medicines sent to an after-hours community pharmacy must comply with the [WA Medicines and Poisons Regulations 2016](#) Part 4, Division 1.10.

S4Rs & S8s transferred from other wards or obtained from a community pharmacy must be entered into the appropriate register.

Refer also to the [NMHS MH Medication: Prescribing and Supply of Stimulants; WA Medicines Handling Policy MP 0139/20](#).

7. Administering

7.1 S8 Medicines Administration

To reduce the risk of diversion, administration of S8 medication requires two authorised staff members to perform independent double-checking, both of whom sign the S8 register. The same

two staff members must also be involved in the administration of the S8 medicine to the patient, both signing the medication chart.

7.2 S4R Medicines Administration

Administration of S4R medication requires two authorised staff members, both of whom must sign the S4R register. The same two staff members must also be involved in the administration of the S4R medicine to the patient, both signing the medication chart.

Refer to [NMHS Administration of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy and Procedure](#).

[WA Guideline on administration and record keeping for Schedule 4 Restricted and Schedule 8 medicines](#).

7.3 Dose Administration Aids (DAA)

Inpatients on the Medication Self-Administration Program require a DAA, usually a blister-pack (e.g. Webster-Pak®), as an integral part of the patient's rehabilitation process.

S8 medicines **must not be** packed in DAAs for use on the ward.

S4R medicines may be packed.

DAAs containing S4R medications are to be stored in the same way as other DAAs. The S4R register is not used for receipt or administration of S4R medication in DAAs.

The DAAs signing sheet is signed and the patient's medication chart is documented with 'S' – self-administered, under the supervision of an approved staff member.

8. Dispensing and Supply

Pharmacy Department: pharmacy staff will dispense discharge and leave prescriptions when required, with the pharmacist performing the final check. Any controlled medicines required the procedures in this policy must be followed. Discharge prescriptions may be sent to a patient's community pharmacy for blister-packing if required.

Doctor dispensing: this must only occur outside the Pharmacy Department hours and for brief periods of unexpected leave; a prescription must be supplied to the hospital pharmacy as a record. Refer to the [NMHS MH Medication: Prescribing, Administration and Management for Inpatients Policy and Procedure](#) (Appendix 3). Discharge medication if required the doctor will provide the patient with a prescription and a voucher for dispensing at a community pharmacy. If S4R and/or S8 medicines are required, the procedures in this policy must be followed.

9. Inventory Management

9.1 Clinical Areas

NMHS MH inpatient sites checking of the S8 medicine balances and register will be performed when the S8 safe key changes hands at least once every 24 hours by the two authorised staff members relinquishing and receiving the S8 key.

Checking of the S4R medicine balances and register on the ward will be performed when the S4R cupboard key changes hands at least once every 24 hours by two authorised staff.

More frequent checking may be required, for example on delivery of S8 or S4R medicines to the ward.

Sealed containers **MUST NOT** be opened to check contents. If a full container is supplied, it will be sealed with a tamper-evident seal. If the seal is broken the container must be opened and the contents must be checked.

When checking medicines in blister strips **BOTH** sides must be checked for tampering.

Two authorised staff members are required for the inventory checks, the same staff must also perform the documentation.

The inventory check must confirm:

- medication formulation and strength,
- correct register balance of stock,
- ensuring that all transactions after the previous check have been completed correctly,
- ensure any balance transfers to a new page or new book have been documented correctly,
- expiry of medication,
- packaging must be checked for evidence of damage or tampering,
- reporting any discrepancies to CNS/coordinator.

Documentation in **red** includes:

- date of inventory,
- 'Checked & Correct',
- printed names and signatures of the two staff members.

For further information refer to [NMHS Inventory of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy](#).

Refer to [Appendix 1](#). Management of S8 and S4R Medicines at Ward Level.

9.2 Pharmacy Department

The S8 controlled medicine balances in the ADM are to be checked once a month using the iPharmacy stocktake sheet by two pharmacists. Monthly iPharmacy reports are forwarded to the Department of Health Medicines and Poisons Regulation Branch by the DCP or Chief Pharmacist. Stock levels when any medication is dispensed, distributed or received must also be checked, this is part of the ADM process.

S4R controlled medicine balances are checked each week by one pharmacist and a pharmacist assistant (or two pharmacists) using the iPharmacy stocktake sheet.

9.3 Controlled Liquid Medicines

Controlled liquid medicines inventory is by a visual inspection of the bottle and estimation of the volume. Physical measurement is conducted at the end of the bottle reconciliation. This avoids loss of liquid with repeated measuring. Doses of liquid medicines will be measured using oral dosing syringes.

The balance recorded in the register will be obtained by subtracting the dose measured from the previous balance and performing a visual inspection.

DO NOT POUR THE CONTENTS OF THE BOTTLE OUT TO CHECK THE VOLUME.

Once the final amount is drawn up from the bottle the balance is reconciled. The two nurses will write the correct volume in the register and sign checked and correct.

THIS MAY BE DIFFERENT FROM THE CALCULATED VOLUME.

9.3.1 Calculating a controlled liquid discrepancy

To calculate the discrepancy per dose:

- count the number of doses since the last reconciliation (check) was performed,
- divide the total volume of the discrepancy (in mL) by the number of doses,
- record the discrepancy per dose as part of the register entry at reconciliation.

Note: Where oral liquid medicines discrepancies are within the 0.2 mL/dose allowance, these are considered resolved, however, at NMHS these need to be reported (only requires Step 1 – Part 1 of the Medicines Discrepancy Form).

Refer to Appendix 3 in the [NMHS Discrepancy Management of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy](#).

Methadone oral liquid for the Community Program for Opioid Pharmacotherapy (CPOP) patients is supplied in pods or as individual measured doses, sealed, labelled with patient's name and the dose for individual patients by the supplying pharmacy department.

Please refer to:

[WA Medicines Handling Policy](#)

[WA Guideline on oral liquid Schedule 4 Restricted and Schedule 8 Medicines.](#)

9.4 Emergency Drug Box/ Medicine Kit

The emergency drug box/ medicine kit contains S4R medicines and must be checked by an authorised staff member:

- the box is present,
- tamper-evident seal is intact,
- date of check,
- name of authorised staff member checking.

Boxes on the ward, must be checked and documented by an authorised staff member, daily to reduce the risk of diversion; NMHS MH Hospital Pharmacy by authorised pharmacy staff, weekly.

If the seal is broken or contents in the box used or expired, the local site Pharmacy department must be notified to arrange collection and a replacement box. The emergency drug trolley in SCGH mental health wards is exchanged at operating theatres and the emergency drug box in Osborne Lodge is exchanged at OPH Pharmacy.

9.5 Patient's Own Medication (POM)

S8 or S4R medicines brought in by patients/ carers family must be counted by an authorised staff member and witnessed by another; documented and placed in a tamper-evident bag, to reduce the risk of diversion. If the POMs remain on the ward, they must be checked daily by two authorised staff members. Document that the S8/S4R POM tamper-evident bag is intact, and the unique bag number is correct in the appropriate controlled medicine register.

Refer to [NMHS Management of Patients' Own Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines](#); [NMHS MH Medication: Patient's Own Medications Policy and Procedure](#).

Refer to [WA Guideline on patients' own medicines](#).

Summary of integrity checks according to area and medication type

Area	Medication Type	Minimum Frequency	Approved Staff
Clinical areas	Medicines kits containing S4Rs (e.g. Emergency Drug Box)	Once a day (excluding days when the clinical area is closed)	1 approved staff
	S8/S4R POMs	Once a day (excluding days when the clinical area is closed)	2 approved staff
Pharmacy areas	Medicines kits containing S4Rs	Once a week	2 approved staff
	S8 POMs	Once a month	2 approved staff
	S4R POMs	Once a week	2 approved staff

Extract from [NMHS Inventory of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy \(11/11/2021\)](#).

9.6 Discrepancies

All approved staff members are required to report identified discrepancies of S8 and S4R medicines to the NMHS Integrity Directorate. This is done via [SHERLOC](#) Controlled Medication Discrepancy Program. For sites where the [SHERLOC](#) program has not been implemented, use the [Schedule 8 and restricted Schedule 4 Medicine discrepancy/loss report form](#).

Pharmacy department discrepancies must be reported to the Chief Pharmacist or DCP.

Any ward discrepancies must be reported to the CNS/coordinator.

Refer to:

[NMHS Discrepancy Management of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines](#).

[WA Reporting of Schedule 4 Restricted and Schedule 8 Medicines Discrepancies Policy](#).

10. Destruction, Discard and Disposal

Medication will be disposed of in the appropriate secure sharps clinical waste (yellow)/ secure pharmaceutical waste (red) bins. **UNDER NO CIRCUMSTANCES WILL MEDICATION BE DISCARDED IN THE SINK.**

Unused, unwanted or expired S8 and S4R medication must be returned to the pharmacy for destruction. Contact the relevant ward pharmacist to arrange collection.

- Keep unwanted medication in the locked cupboard/safe until collected.
- On collection, the nurse and pharmacy staff member will seal the medication into a tamper-evident bag and document the unique bag identifier on the requisition.
- The Nurse and pharmacy staff member will record the return in the S8 or S4R register, including the requisition number in the entry.

Waste produced from the preparation of a single dose, such as partial tablets, part ampoules, used patches and refused doses may be discarded on the ward.

- Discards of S8 and S4R medication must be performed by two authorised staff members, to reduce the risk of diversion. The witness needs to:
 - Verify the medicine label.
 - Verify the amount to be discarded matches the documentation.
 - Observe the disposal of the medicine into the appropriate waste container.
- Tablets, patches and liquids will be discarded in the secure sharps clinical waste (yellow) / secure pharmaceutical waste (red) bins which are stored in the medication storage rooms.
- Transdermal patches that have been removed from patients' skin will be folded so the adhesive sides stick together and cut into small pieces before discarding in the yellow/ red bin. [NMHS MH Medication: Transdermal Patch Management](#).
- In the case of ampoules, vials or discontinued infusions, the entire contents must be emptied into the yellow/red bin. The empty containers should be disposed of in the same bin.
 - Where part of a tablet or vial is discarded in the preparation of a dose, the discard is recorded in the relevant register in the 'amount discarded' column.
 - For all other discards, the reason for disposal is recorded and signed in the back of the S8 or S4R register.
 - Medicines that are accidentally damaged and unusable (e.g. dropped and broken ampoules – instances where return to pharmacy is impractical) and witnessed by two staff will be discarded in accordance with this procedure.
 - All documentation must be legible, signatures identifiable, names in full, completed in full in a timely manner, in indelible ink and not altered.

Pharmacy will destroy expired stock; stock exposed to compromised conditions; stock not suitable to return to pharmacy inventory or unwanted Patient's Own Medication. Pharmacy will refer to the [NMHS Destruction, Discard and Disposal of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy](#) and [Procedure](#).

Summary of Destruction/Disposal Processes

Formulation	Process
Tablets	Full/partial tablets must be crushed (use a tablet crusher/mortar & pestle) & dispersed in a small volume water prior to disposal. Note: Some slow-release (SR) formulations are difficult to crush & may require soaking in water prior to crushing. If experiencing particular difficulty crushing SR formulations, these may be disposed of in a secure pharmaceutical or secure clinical waste bin for incineration.
Capsules	Must be opened & contents dispersed in a small volume water prior to disposal. The empty capsule may be disposed of in a general waste bin.
Oral liquids	Liquid contents must be poured into the secure sharps clinical waste (yellow) / secure pharmaceutical waste (red) bins.
Lozenges and films	Disperse in a small volume of water (hot water if possible) prior to disposal.
Suppositories & pessaries	Disperse in water (hot water if possible) and dissolve prior to disposal
Patches/transdermal formulations	Use gloves to avoid inadvertent contact of the patch and skin. Fold patch onto itself so the adhesive sides stick together, cut into small pieces with scissors & place in a secure sharps clinical waste or secure pharmaceutical waste bin.
Ampoules and vials	Entire contents must be drawn out (e.g. with a needle & syringe) - expelled into the secure sharps clinical waste (yellow) / secure pharmaceutical waste (red) bins.
Syringe	Entire contents must be expelled into the secure sharps clinical waste (yellow) / secure pharmaceutical waste (red) bins.
Infusion bag	Entire contents must be emptied from the bag into the secure sharps clinical waste (yellow) / secure pharmaceutical waste (red) bins.
CADD®	Entire contents must be drawn out and expelled into the secure sharps clinical waste (yellow) / secure pharmaceutical waste (red) bins.

11. Transport of S8/ S4R Medicines

Chain of custody must be maintained for all S8 and S4R medicines to reduce the risk of diversion. Requirements specific to transport are:

- Authorised Staff.
- Approved methods.
- Tamper-evident packaging.
- Documentation of supply and receipt.

Refer to:

[NMHS Transport of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines](#)

[Appendix 3: Approved Staff and Approved Transport Methods of S8/S4R Medicines.](#)

11.1 Transport of inpatient S8/S4R Medicines – Imprest and Inpatient Supply

- Inpatient/imprest medicines may be transported by:
 - Patient Support Services staff but must be handed directly to a pharmacist or authorised staff member in the clinical area.
 - Pharmacy support staff, they will receive the medicines into the appropriate registers and storage unit with a pharmacist or authorised staff member.
- S8/S4R medicines must be transported in tamper-evident packaging.
- S8/S4R medicines must be signed into the appropriate register on arrival to the intended location by two authorised staff members.
- A delivery docket or requisition slip must be included, signed and returned to the supplying pharmacy department.

11.2 Transferring S8/S4R Medicines between Wards

This may occur after-hours, when a patient transfers wards or there is no stock available in the pharmacy, and the required stock is available on another ward.

Process:

1. The requesting ward's authorised staff member must take the appropriate S8/S4R register to the ward with the medicine stock.
2. Two authorised staff members must sign the S8/S4R out of the supplying ward's appropriate register.
3. Two authorised staff members must sign the S8/S4R into the requesting ward's appropriate register.
4. At the requesting ward, two authorised staff members must check the balance of the stock and sign the appropriate S8/ S4R register.
5. The pharmacy department should be informed of any transfers.

11.3 Transport of Discharge/Leave S8/S4R Medicines

- Discharge/leave S8/S4R medicines:
 - can be transported by approved staff,
 - must be transported in tamper-evident packaging (may also contained non S8/S4R medicines),
 - on receipt by the ward, must be signed into a manual log or register on the ward and witnessed by authorised staff members,
 - on giving to a patient or carer, must be signed out of the log or register by an authorised staff member and the patient or carer.

11.4 Transport of S8/S4R Patient's Own Medicines (POMs)

POMs can be transported by authorised staff or pharmacy staff:

- POMs containing S8 or S4R medicines must be placed in an appropriate POM S8/ S4R tamper-evident bag with a unique serial number.
- Two approved staff members must sign the POM out of appropriate register, documenting the area it will be transferred to e.g. pharmacy or another ward.
- The POM must be accompanied with a Statement of Patient's Own Medication (SPOM) form.
- At receiving area two approved staff members sign the POM into the appropriate register.
- Refer to [NMHS MH Patient's Own Medication Policy](#).

11.5 Transport of Emergency Medicine Kits Containing S4Rs

Emergency Medicine Kits (or Emergency Drug Box):

- Must have tamper-evident seals attached,
- Can be transported by approved staff members.

11.6 Transport of S8/S4R Outpatient Medicines

Within the same NMHS site: transported by any approved staff.

Outside NMHS site:

- contracted couriers must be used, not taxis or registered Australia Post.

11.7 Transport of S8/S4R Medicines to/from other sites (Borrow/Lend)

NMHS MH Pharmacy Department must facilitate transport of all S8/S4R medicines which occur outside of the same NMHS site.

Couriers or an authorised staff member must be used for the transport of S8/S4R borrow/lends outside the NMHS site:

- Taxis or registered Australia Post must **not** be used for transport of S8/S4R borrow/lends.
- Contracted couriers who can ensure safe, secure and timely delivery of S8/S4R medicines must be used for transport of these medicines.
- Contracted couriers must keep vehicles secured when unattended and have procedures for managing incidents during transit (e.g. road accidents involving the courier vehicle).
- Confirmation of the receipt by the other entity of S8/S4R borrow/lends must be available upon request from the courier.

When transporting (lending) S8/S4R medicines to another site, these medicines must be packaged in tamper-evident packaging which does not indicate contents within the package.

12. Day Procedure Unit (DPU)

S8 and S4R medications used in DPU are stored in the ward S8 and S4R cupboards together with a separate register and are checked as part of the ward S8 and S4R check.

At each treatment session the containers and registers are taken to the DPU by two authorised staff members attending DPU from the ward.

Any medication used is entered in the register by the DPU authorised staff member accompanying the patient.

At the conclusion of the treatment session the balance of all DPU S8 and S4R medications remaining in the ward S8 and S4R cupboards is checked by the DPU authorised staff member along with an authorised staff member from the ward, the register annotated.

13. Exceptions to Administration/Discard in Specific Areas or Situation

An exception for the requirement for an independent witness to discards of S4R and S8 medicines is made for anaesthetists in Theatre and procedural areas. Refer to [NMHS Destruction, Discard and Disposal of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy and Procedure](#).

The anaesthetist accountable for the clinical care of the patient is the person responsible for discarding any used S4R or S8 medications utilised during the procedure, including intravenous infusions.

14. Compliance and Evaluation

Compliance:

- Ward:
 - Stocktake check of S8 medicines in safe.
 - S4R medicines in cupboard against registers at least once every 24 hours.
- Pharmacy:
 - Stocktake check of S8 safe and registers monthly.
 - Stocktake check of S4R drawers weekly.

NMHS Audits as contained on the [NMHS Medicines Management](#) intranet page.

Refer to [Appendix 2](#) NMHS Medicines Management Audit Schedule for Clinical and Pharmacy Areas.

Related internal policies, procedures and guidelines
NMHS Administration of Schedule 8 (S8) and Schedule 4 Restricted (S4R) Medicines Policy and Procedure
NMHS Order and Receipt of Schedule 8 (S8) and Schedule 4 Restricted (S4R) Medicines Policy
NMHS Supply and Distribution of Schedule 8 (S8) and Schedule 4 Restricted (S4R) Medicines Policy

[NMHS Destruction, Discard and Disposal of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy and Procedure](#)

[NMHS Discrepancy Management of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines](#)

[NMHS Inventory of Schedule 4 Restricted \(S4R\) and Schedule 8 \(S8\) Medicines Policy](#)

[NMHS MH Medication: Transdermal Patch Management](#)

[NMHS Medication: Management of Patient's Own Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy](#)

[NMHS MH Medication: Patient's Own Medications](#)

References

[Standard for the Uniform Scheduling of Medicines and Poisons \(SUSMP\)](#)

[WA Medicines and Poisons Act 2014](#)

[WA Medicines and Poisons Regulations 2016](#)

[WA Medicines Handling Policy](#)

[WA Risk based requirements for medicines handling](#)

[WA Guideline on administration and record keeping for Schedule 4 Restricted and Schedule 8 medicines](#)

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NSQHS Standards Applicable:	☒  Std 1: Clinical Governance		☒  Std 4: Medication Safety		
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 8: Governance, leadership and management				
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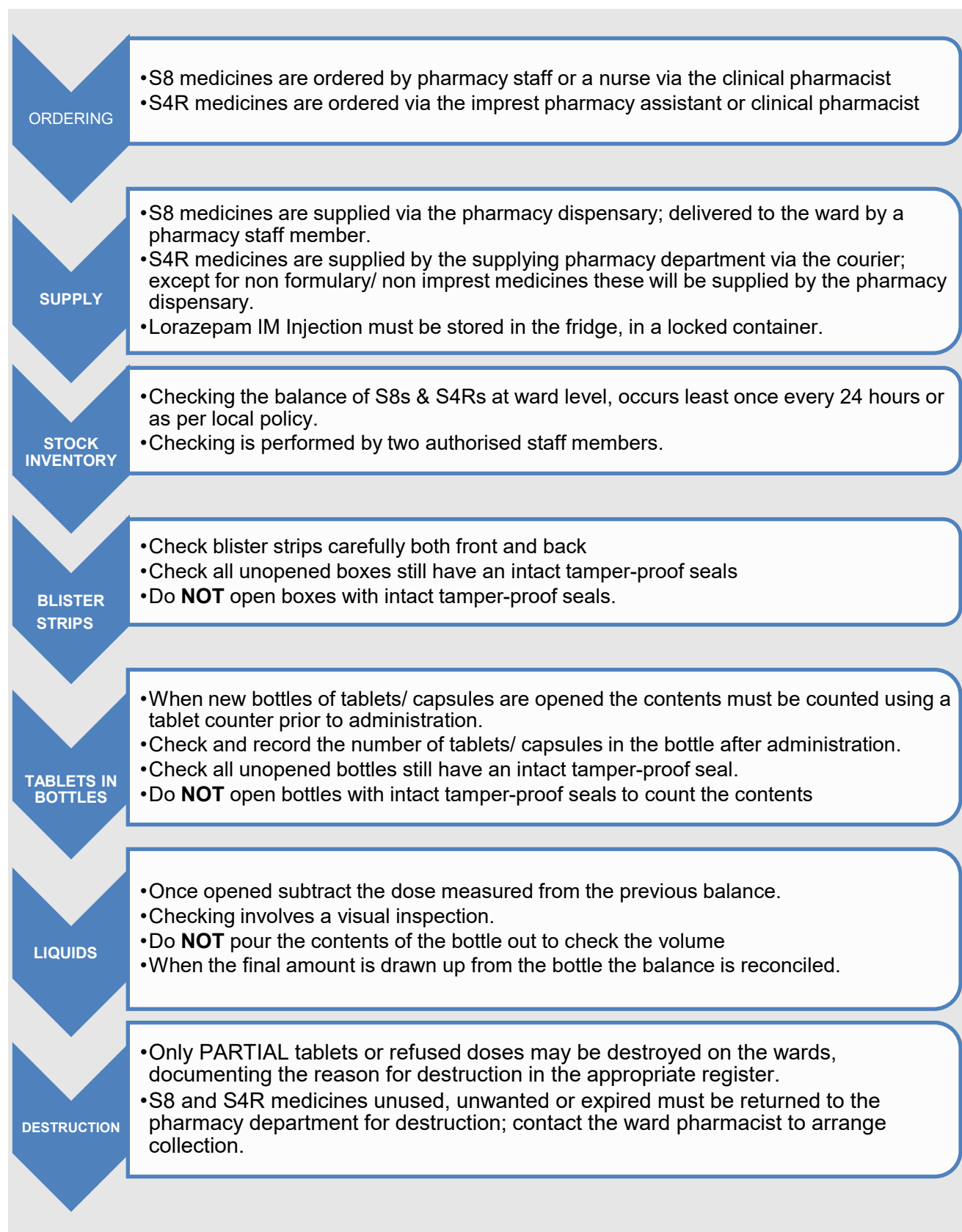
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Date	Version	Author	Notes
11/05/2022	V1	NMHS MH Chief Pharmacist	New policy/ procedure. Information removed from the MH medicine management policy. Includes NMHS Medicine Management Policies for controlled medicines and WA Medicines Handling policy. NMHS audit schedule.
21/02/2023	V1.1	NMHS MH Chief Pharmacist	Inclusion of NMHS Transport of Controlled Medicines; changes to S8 administration; References added; Extra information added to Appendix 2. Exemption Approval
16/10/2023	V2	NMHS MH Medication Safety Pharmacy	Amendments to remove barriers to EN scope of practice
16/01/2024	V2.1	NMHS MH Chief Pharmacist	Updated to reflect changes to include EN as an authorised person.
May 2024	V2.2	NMHS MH Chief Pharmacist	Updated telephone orders for S8s; include S8 pod information; hyperlink for NMHS MH Patient's Own Medication Policy & Procedure; Information on lost keys; automated dispensing machine.
26/06/2025	V3.0	NMHS MH Chief Pharmacist	Major update to comply with KPMG external audit results.

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Appendix 1. Management of S8 and S4R Medicines at Ward Level



Appendix 2. NMHS Medicines Management Audit Schedule

Clinical Areas (Nursing Staff)

Audit	Jan	Feb	Mar	Apr	May	Jun	Jul	Aug	Sep	Oct	Nov	Dec
Discrepancy Management (by NMHS)												

Pharmacy Areas (Pharmacy Staff)

Audit	Jan	Feb	Mar	Apr	May	Jun	Jul	Aug	Sep	Oct	Nov	Dec
Discrepancy Management (NMHS)												

Appendix 3: Approved Staff and Approved Transport Methods of S8/S4R Medicines

	Nurses	Medical Staff	Pharmacists	Pharmacy Support Staff	Patient Support Services Staff	Anaesthetic Technicians	Courier
Inpatient/impresst S8/S4R medicines	Yes	Yes	Yes	Yes	Yes	Yes	N/A
Inpatient S8/S4R medicines (inter-ward transfer)	Yes	Yes	Yes	Yes	No	N/A	N/A
S8/S4R Discharge/Leave Medicines	Yes	Yes	Yes	Yes	Yes	N/A	N/A
Medicines Kits containing S4Rs	Yes	Yes	Yes	Yes	Yes	Yes	N/A
S8/S4R Outpatient Medicines	Yes	Yes	Yes	Yes	Yes	N/A	Yes
S8/S4R POMs	Yes	Yes	Yes	Yes	No	N/A	Yes
Transport to other sites (borrow/lend)	N/A	N/A	Yes	Yes	N/A	N/A	Yes
Inpatient S8/S4R medicines (intra-procedural area transfer)	Yes	Yes	Yes	Yes	No	Yes	N/A

Appendix 4 Staff Authorisations for handling of S8 and S4R Medicines

Handling of Schedule 8 (S8) and Schedule 4 Restricted (S4R) Medicines									
STAFF		Prescribe	Administer	Dispense	Supply	Transport	Discard	Destruction	
AUTHORISED STAFF	APPROVED STAFF	Medical Practitioners	Y	Y		Y	Y	Y	Y
		Nurse Practitioners	Y	Y		Y	Y	Y	Y
		Dentists	Y	Y		Y	Y	Y	Y
		Endorsed Midwives	Y	Y		Y	Y	Y	Y
		Endorsed Podiatrists	Y	Y		Y	Y	Y	Y
		Anaesthetic Technicians		Y			Y	Y	
		Pharmacists			Y		Y		Y
		Registered Nurses		Y			Y	Y	Y
		Enrolled Nurses		Y			Y	Y	Y
		Midwives		Y			Y	Y	Y
		Student nurses/midwives under direct supervision of a registered nurse/midwife		Y			Y	Y	
		Pharmacy Technicians					Y		
		Patient Support Services					Y		
		Contracted Couriers					Y		
IMPORTANT NOTES: INDIVIDUAL STAFF MUST ALSO CONSIDER THEIR PERSONAL SCOPE OF PRACTICE AND PROFESSIONAL CREDENTIALING IN DETERMINING WHETHER THEIR HANDLING OF S8/S4R IS “THE LAWFUL PRACTICE OF THEIR PROFESSION”.									